

Urinary Diversion: Comparing Your Options

Patient Information · Three ways to drain urine when the bladder is removed or no longer works

WARWIKI

When the bladder is removed or no longer works, urine needs a new way to leave the body. This is called urinary diversion. There are three main types. Each uses a piece of your own bowel. They differ most in **how you empty urine each day**: into a bag on the belly, through the urethra, or with a catheter. Each type also has its own handout.

The Three Options

WEARS A BAG

Ileal Conduit

A short piece of bowel carries urine to a small opening (a **stoma**) on the belly. Urine drains all the time into a light **bag** that you empty and change. It is the simplest and most common type. There is nothing to catheterize.

URINATES THROUGH THE URETHRA · NO BAG

Orthotopic Neobladder

A new pouch made from bowel is joined to your **urethra**, so you urinate the usual way with no bag. You learn to empty on a **schedule**, day and night. Some people need a catheter to empty it. Leaks, mostly at night, are common at first.

EMPTIES WITH A CATHETER · NO BAG

Continent Cutaneous (Indiana Pouch)

A pouch made from bowel stores urine inside you. A small opening on the belly stays closed between uses. There is no bag. You pass a **catheter** through the opening to empty it several times a day.

For some people who should not have bowel surgery, a simpler option joins the ureters straight to the skin (a cutaneous ureterostomy).

Learn the Terms

Urinary diversion

A new path for urine to leave the body when the bladder is gone or not working.

Stoma

A small opening on the skin that urine passes through.

Conduit

A short piece of bowel that carries urine to a stoma. It does not store urine.

Neobladder

A new bladder built from bowel and joined to the urethra.

Continent

Does not leak. You choose when it empties.

Ureters

The tubes that carry urine from the kidneys.

Catheterize

To pass a thin tube to drain urine.

Mucus

Slippery fluid the bowel makes. Strands in the urine are normal after any of these.

Which is right for me? No one type is best for everyone. The choice depends on your kidney and liver function, your bowel, your other health problems, and whether you can catheterize (this takes steady hands and good eyesight). It also depends on what matters most to you. Not everyone can have every type. Your surgeon will tell you which are safe for you.

How They Compare

	Ileal Conduit	Neobladder	Continent (Indiana)
How you empty	Drains on its own	Urinate through the urethra	Catheter through the stoma
Wear a bag?	Yes	No	No
Catheter needed?	No	Sometimes	Always, on a schedule
Surgery size	Smallest	Larger	Larger
Daily effort	Empty and change the bag	Timed voiding, day and night	Catheterize and rinse

Questions to Ask Your Surgeon

- Which types can I have, and which do you suggest for me?
- What will **daily life** and self-care look like with each?
- How will each affect **work, exercise, sleep and sex**?
- What are the main **risks**, and how likely am I to need another procedure later?

The Same for All Three

- A piece of your **own bowel** is used, so the bowel needs a few days to start working again after surgery.
- **Mucus** in the urine is normal and lasts for life.
- Problems can show up years later, sometimes with no symptoms. So you need **check-ups for life**: blood tests (kidney function, acid level, vitamin B12) and kidney scans.
- **Drink plenty of fluids.**

Three Things to Remember

1. There are three main types: a **bag** (ileal conduit), urinating through the **urethra** (neobladder), or a **catheter** through a small opening (Indiana pouch).
2. They differ most in **daily care**. The best fit depends on your health and what matters to you.
3. Every type needs check-ups for life. Read the handout for the type you choose.